



Pharm*Achieve*

MEDICAL ABORTION

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

**Providing Care:
Clinical Care**

**Providing Care:
Distribution**

**Knowledge
& Expertise**

**Communication
& Collaboration**

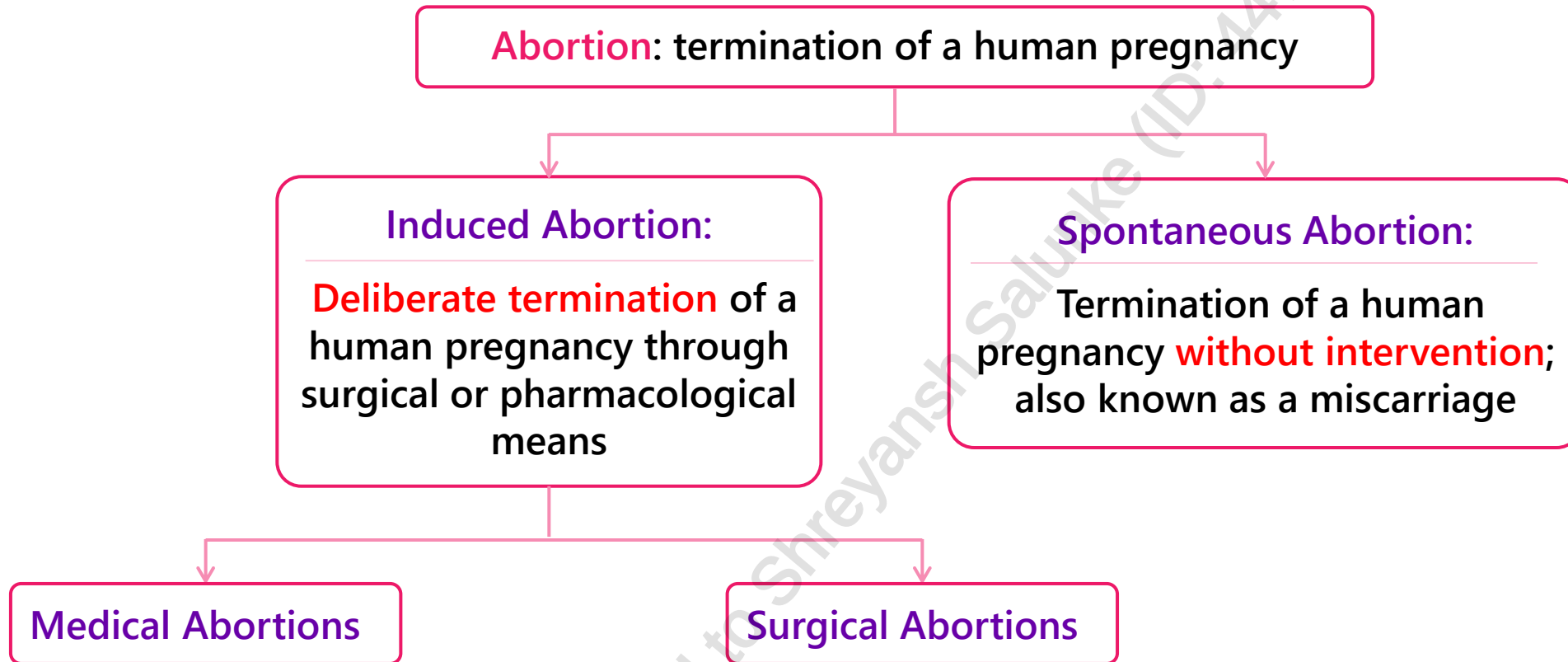
**Leadership &
Stewardship**

Professionalism

LEGEND

ADR	Adverse Drug Reaction
CI	Contraindication
CS	Corticosteroids
Cu-IUD	Copper Intrauterine Device
CYP	Cytochrome P450
Hb	Hemoglobin
hCG	Human Chorionic Gonadotropin
HR	Heart Rate
IBD	Inflammatory Bowel Disease
IM	Intramuscular
IUD	Intrauterine Device
IUS	Intrauterine System
MA	Medical Abortion
MD	Doctor of Medicine
NSAIDs	Non-Steroidal Anti-inflammatory Drugs
PO	Per Os (Oral)
PRN	Pro Re Nata (As Needed)
RPh	Registered Pharmacist
Rx	Prescription
SL	Sublingual
STIs	Sexually Transmitted Infections

INTRODUCTION



- 50% of unplanned pregnancies end in abortions
- Induced abortions have been **legal in Canada since 1969**
- **Mifepristone/misoprostol approved in 2015** by Health Canada

MEDICAL ABORTION ASSESSMENT

FYI

Confirm pregnancy

beta-hCG urine test

Medical History

CIs, medications, STIs, psychosocial history

Physical Exam

Height / Weight / Vitals / Pelvic Exam

Ultrasound *gold standard but not mandatory*

Laboratory testing

CONTRAINDICATIONS FOR ALL MEDICAL ABORTION

Ectopic pregnancy

IUD in situ

Bleeding disorders

Anemia
(Hb <95 g/L)

Drug allergies

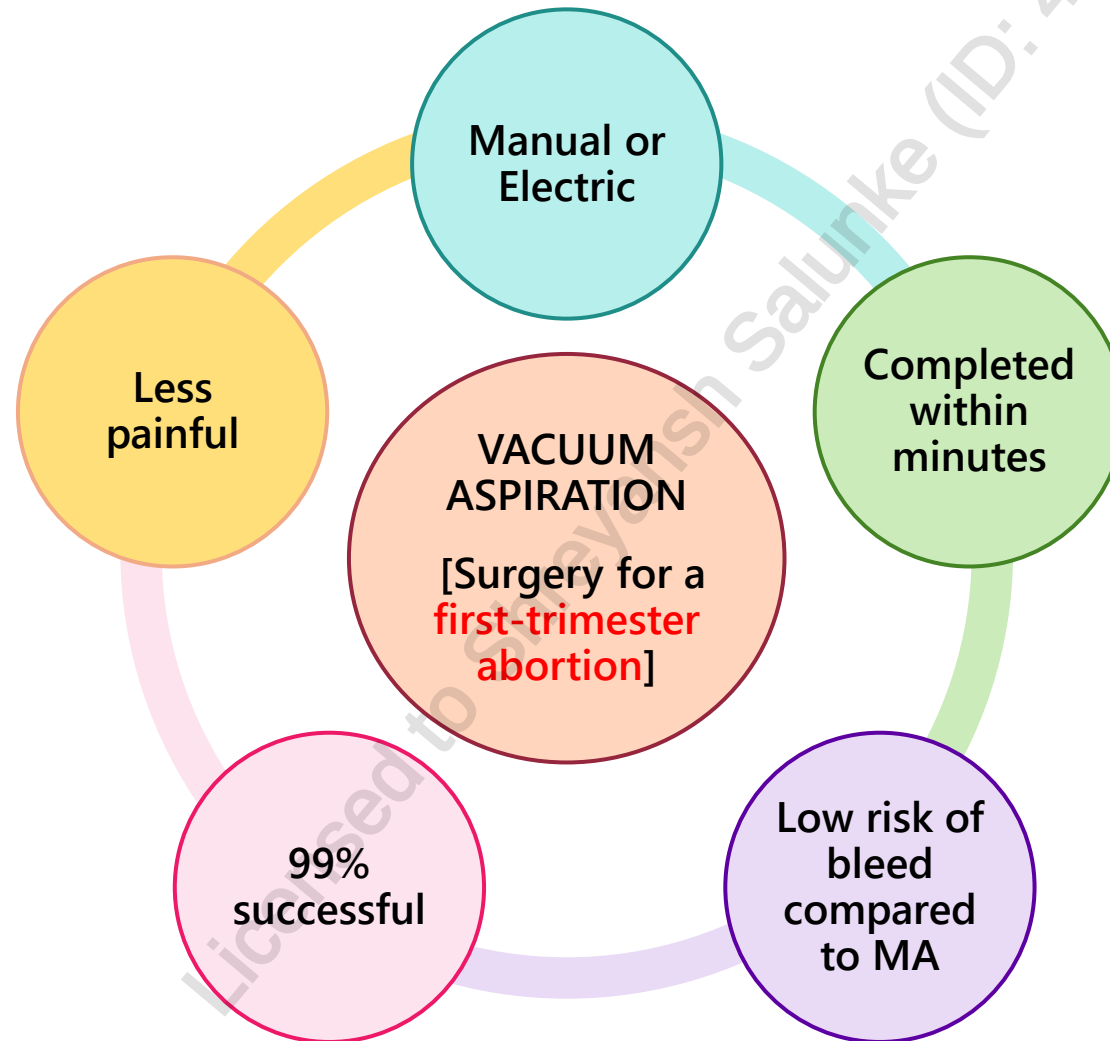
Ambivalence

Chronic adrenal
failure

Chronic systemic
CS

Uncontrolled
asthma

NON-PHARMACOLOGICAL MEASURES



PHARMACOLOGICAL MEASURES

MIFEPRISTONE

- Progesterone receptor modulator

MISOPROSTOL

- Prostaglandin analogue

Combination therapy is Health Canada approved for use at ≤ 63 days gestation



Important for RPh to check date on Rx

If pt brings the Rx 7 days later than issued → contact MD

- Day 1: 200 mg of mifepristone taken PO as 1 tablet
- Day 2 or 3: 800 mcg of misoprostol taken as 4 x 200 mcg
 - Suggested administration for misoprostol: buccal tablets placed between the cheek and gums on each side of the mouth. Leave for 30 min then swallow remaining pieces
- Misoprostol tablets can be taken buccal, vaginal, or SL
- **Interactions from mifepristone:** CYP 3A4 inducers could cause treatment failure. Inhibitors can increase ADRs
 - Mifepristone is metabolized by CYP3A4, but it is also a competitive inhibitor of the enzyme

ALTERNATIVE THERAPY

FYI

Methotrexate and misoprostol

- Off-label use
- Day 1: 50 mg methotrexate PO or IM
- Day 4, 5, or 6: 800 mcg of 4 x 200 mcg vaginal tablets

Misoprostol

- 800 mcg of 4 x 200 mcg vaginal/sublingual tablets
- Much less effective than mifepristone/misoprostol
- May require multiple doses

Only used if mifepristone is contraindicated or cannot be accessed!

MANAGEMENT OF SIDE EFFECTS

BLEEDING

- **Day 2 (misoprostol):** Within 4 hours of misoprostol or within 48 hours
- More bleeding than what is experienced during menstruation
- Can include clots and tissue
- Use full-sized sanitary pads; **not tampons**
- Very light bleeding can last up till 2 weeks after medical abortion is complete [can use liners]
- **Seek medical attention if:**
 - Soaking ≥ 2 pads/hr for 2 hours
 - Soaking 1 pad/hr for 10 hours
 - Experiencing **dizziness, light-headedness, tachycardia, analgesics unable to control pain or foul-smelling discharge**

CRAMPING/PAIN

- Starts 2-4 hours after misoprostol administration and should resolve within 24 hours
- **NSAIDs (ibuprofen or naproxen) PRN** are first line
- **Opioids (codeine/acetaminophen or oxycodone/acetaminophen)** are second line
- **Seek medical attention, if the pain is very severe and uncontrolled by analgesics**

PROSTOGLANDIN EFFECTS

- **Self-limiting** symptoms of nausea, vomiting, diarrhea, headache, dizziness, chills, and fever
- Can use **antiemetics** for nausea
- Can use **loperamide** for diarrhea
- **Seek medical attention if these symptoms continue 24 hours post misoprostol administration**

COMPLICATIONS

INCOMPLETE ABORTION

- Suspect ongoing pregnancy **if no bleeding**
- **Teratogenicity** risks with misoprostol
- **Fetal malformations** with methotrexate
- Limited data on mifepristone

PELVIC INFECTION

- Rates of infection **very low**
- **Prophylactic antibiotics are not recommended in medical abortion**
- Mild infections can be treated with **broad-spectrum antibiotics**
- Severe infections will require hospitalization

TOXIC SHOCK SYNDROME

- **Very rare**
- Fast onset and is usually **fatal**
- **Antibiotic treatment** & surgical debridement
- **Hysterectomy** may be required
- Symptoms: Swelling, increase in HR, feeling like you have the flu, no fever, abdominal pain

MONITORING AND FOLLOW-UP

- Follow up should be conducted 7 days after treatment is complete.
- Ensure patient is not experiencing any symptoms of continued pregnancy
- **Beta-hCG levels should decrease rapidly within 24 hours of expulsion**
 - Day 3 follow-up: 50% drop in levels
 - Day 7 follow-up: $\geq 80\%$ drop in levels
- Ultrasound conclusively confirms whether treatment was successful or not
- Patients' fertility can return as early as 8 days post MA:
 - Hormonal contraception can be started on the same day as misoprostol administration
 - Intercourse can be resumed only after MA is complete and tissue has passed.
- IUD/IUS can only be placed after the treatment is complete and successful
- Good resource for practicing pharmacists:

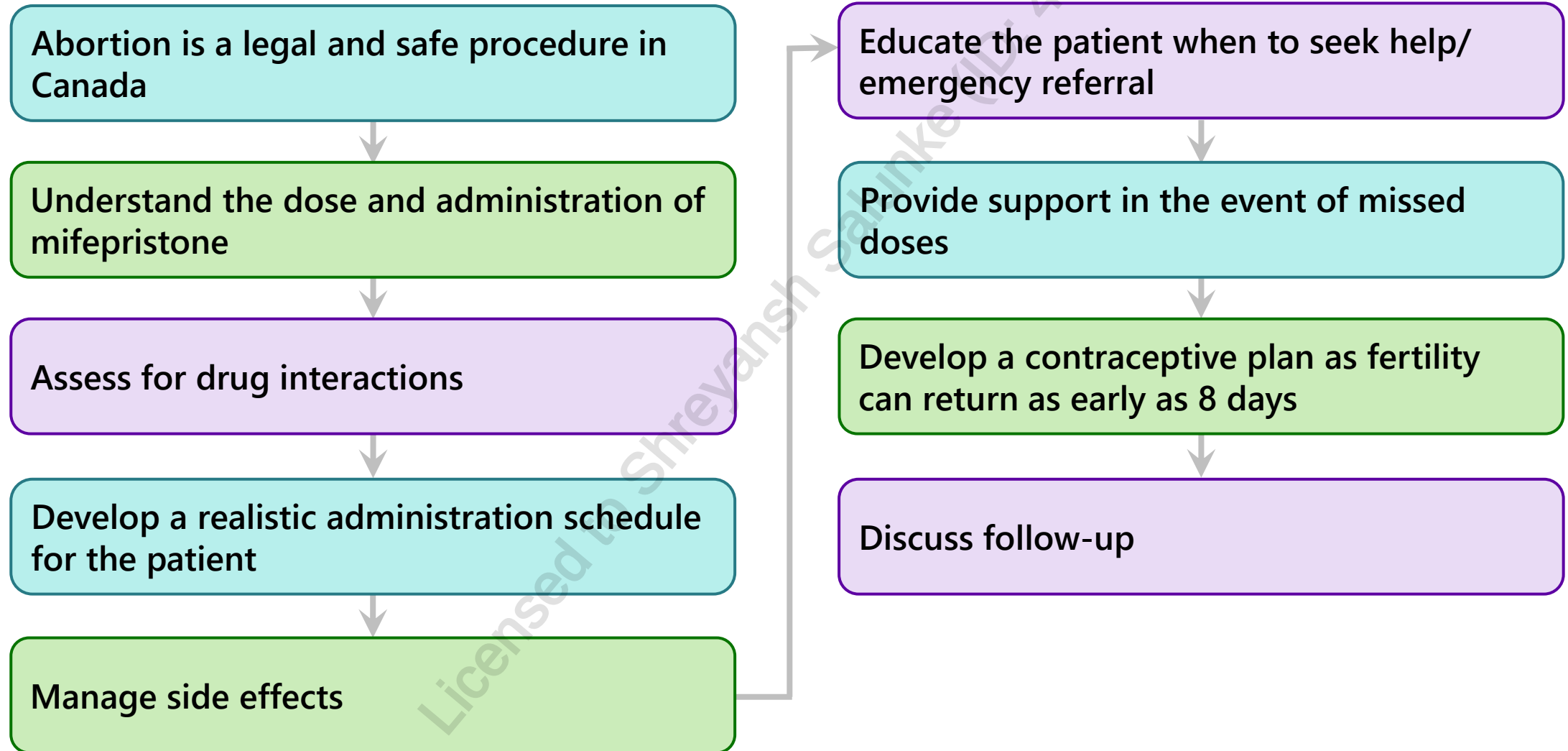


Pharmacist Resource Guide for Medical Abortion



Update on medical abortion

TAKEAWAY



CASE

AR is a 28-year-old, 2nd year medical resident. She presents to your pharmacy with a prescription for Mifegymiso® (mifepristone 200 mg/misoprostol 200 mcg). She has been on-call at the hospital for the past 48 hours and finally had a chance to fill her prescription. AR is married and has one daughter that is 2 years old. She is currently pregnant at 28-days gestation. She would like to terminate the pregnancy. She has 3 days off from work and goes back on call after that. She was using Micronor® (norethisterone 350 mcg PO daily) as a form of contraception, however, due to her busy schedule she occasionally forgets to take her dose on time. Her current medical conditions include migraine with sensitivity to light, type 2 diabetes mellitus and generalized anxiety disorder. Her current medications include acetaminophen 500 mg PO PRN Q4-6H for headaches, nortriptyline 10 mg PO daily, metformin 1000 mg PO BID, alprazolam 0.5 mg PO PRN at night, vitamin D 1000 units PO daily, vitamin C 250 mg PO daily. She also presents a new prescription for clarithromycin (250 mg PO BID for 10 days) for a recent Group A streptococcal infection that has been circulating in the hospital. She enjoys running outdoors and walking with her toddler.

She will drink wine occasionally (1-2 glasses per week) on the days that she is not on call. She tries her best to eat a well-balanced diet, but this can be challenging due to her work schedule.

1. Can we dispense Mifegymiso® for AR today?
 - a) No, because 48 hours have passed since the prescription was written
 - b) No, because of a drug interaction
 - c) Yes, but some counselling is needed
 - d) No, because of a medical condition



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2. Do any of AR's medications interact with Mifegymiso®?

- a) Yes, nortriptyline and clarithromycin
- b) Yes, alprazolam and metformin
- c) Yes, clarithromycin and metformin
- d) Yes, clarithromycin and alprazolam



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3. Please select the FALSE statement:

- a) Patients take 1 tablet of mifepristone and 4 tablets of misoprostol
- b) Mifepristone can be taken on its own, if the patient cannot tolerate misoprostol or methotrexate
- c) Misoprostol can be administered via buccal, sublingual or vaginal route.
- d) According to SOGC guidelines, Mifegymiso® can be used in women up to 70 days gestation



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4. Please select the TRUE statement:

- a) AR should be prescribed Alesse® (ethinylestradiol 20 mcg/levonorgestrel 100 mcg) PO daily during her follow-up appointment to prevent future pregnancy
- a) We do not need to counsel on condom usage, as she is married and not at risk of STIs
- b) Cu-IUD can only be placed once MA is complete
- c) Follow-up should be scheduled 28 days after misoprostol administration



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CHANGE LOG

August 2025

- Formatting, grammar, and spelling changes made throughout
- Slide 3: Added legend
- Slide 5: Added FYI
- Slide 6: Added contraindications
- Slide 8: Removed contraindications as discussed in slide 6, amended note on SL misoprostol administration
- Slide 9: Added FYI
- Slide 18: Updated references

November 2025

- Slide 8: Added that mifepristone is a substrate and a competitive inhibitor of CYP3A4

March 2026

- Slide 2: Updated NAPRA competencies
- Slide 8: Minor wording changes